

Patient ID: WheelerPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 240/720h (33%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

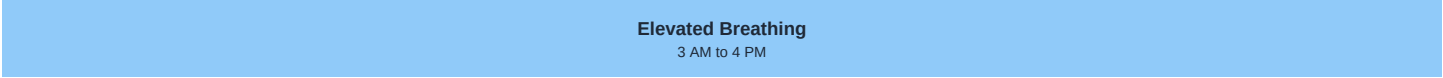
Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours



Vital Stat	Avg	Min	Max
Heart Rate	64 bpm	51 bpm	80 bpm
Breathing	24 brpm	8 brpm	34 brpm

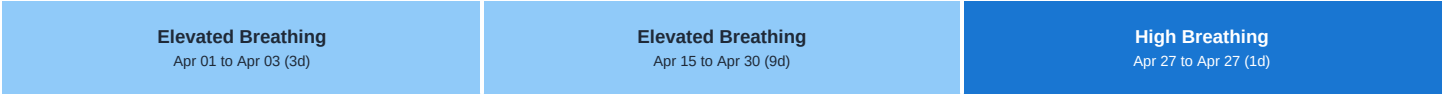
Last 24 hours: two elevated breathing episodes overnight, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
3 AM to 4 PM	14h	2	Elevated Breathing	26	10	34	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 30 days from Apr 01 to Apr 30

■ HR ■ Breathing



28 episodic events Detected, spanning 3 days total. Conditions: Elevated Breathing, High Breathing. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
28	92h	Elevated Breathing	<1	33%

Major Findings: Sustained elevated breathing (avg 26, peak 48)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 27	8 AM to 9 AM	1h	1	High Breathing	30	26	33	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Apr 01	1 AM to 3 PM	32h	8	Elevated Breathing	26	9	48	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 15	9 PM to 10 PM	59h	19	Elevated Breathing	26	9	47	Check SpO2 and lung sounds; assess for fluid overload or early infection

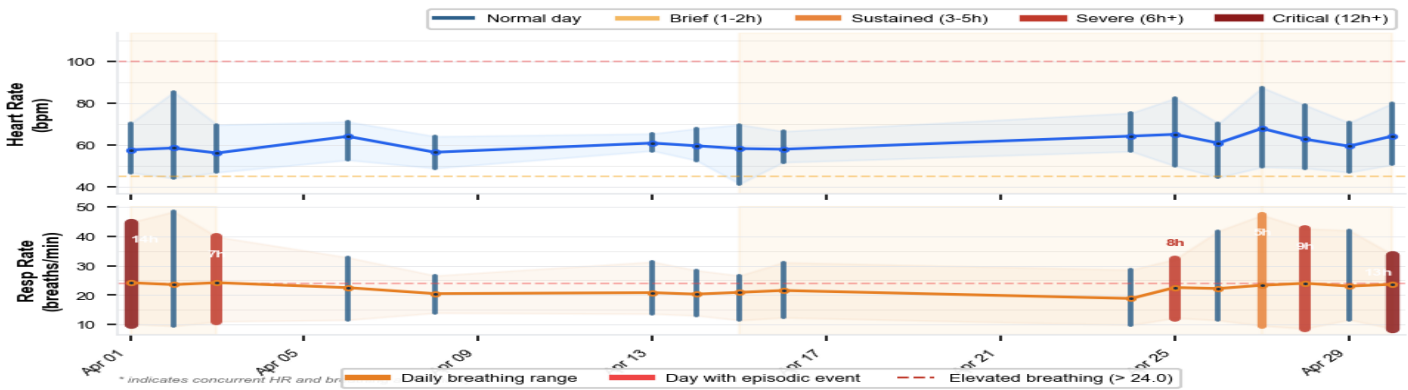
Low data coverage (33%); interpret with caution.

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 47 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 30 brpm, peak 33 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 3 unstable phases with 19 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

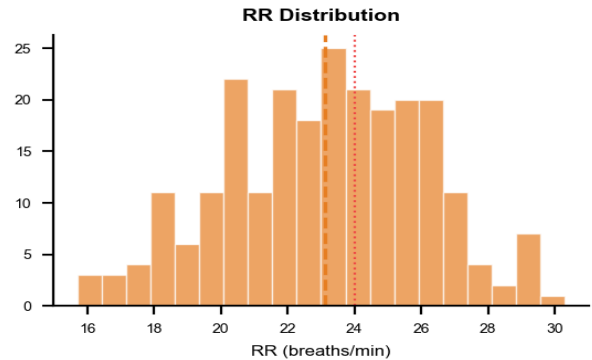
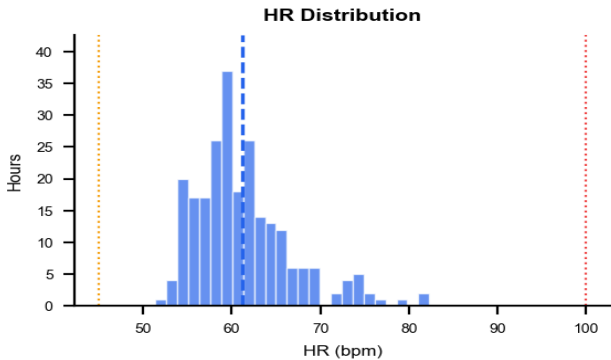
Clinical Pattern Observations:

- 1. Sustained finding: 14h continuous Elevated Breathing on Apr 01.
- 2. Clustered pattern: Episodic events on 6 of 30 days (20%).
- 3. Nocturnal pattern: 3 of 4 eligible episodes during evening or night hours.

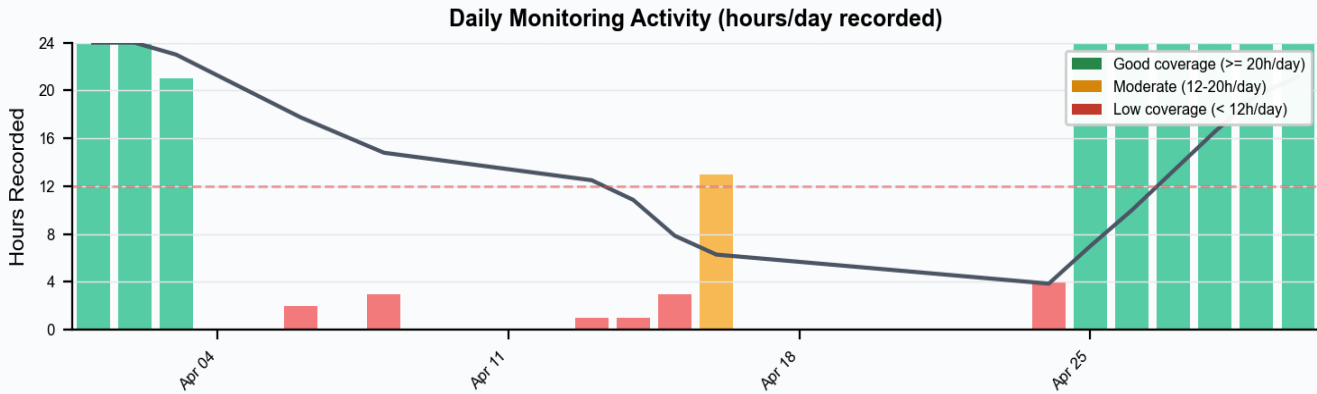


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.